

OLODIN

(Olopatadine Hydrochloride Ophthalmic Solution USP 0.1% w/v)

Description

Clear, colourless solution

Composition

Olopatadine hydrochloride USP equivalent to Olopatadine..... 1.0mg

Benzalkonium Chloride NF (as Preservative) ... 0.1mg

Sterile Aqueous Vehicleq.s

Pharmacodynamics

Olopatadine is a potent selective antiallergic /antihistaminic agent that exerts its effects through multiple distinct mechanism of action. It antagonises histamine (the primary mediator of allergic response in humans) and prevents histamine induced inflammatory cytokine production by human conjunctival epithelial cells. Data from in vitro studies suggest that it may act on human conjunctival mast cells to inhibit the release of pro-inflammatory mediators. In patients with patent nasolacrimal ducts, topical ocular administration of olopatadine was suggested to reduce the nasal sign and symptoms that frequently accompany seasonal allergic conjunctivitis. It does not produce a clinically significant change in pupil diameter.

Pharmacokinetics

Absorption

Olopatadine ophthalmic solution is absorbed systemically, as are other topically administered medicinal products. However, systemic absorption of topically applied olopatadine is minimal with plasma concentration ranging from below the assay quantitation limit (< 0.5ng/ml) up to 1.3 ng/ml. These concentrations are 50-to 200-fold lower than those following well tolerated oral doses.

Elimination

From oral pharmacokinetic studies, the half-life of olopatadine in plasma was approximately eight to 12 hours, and elimination was predominantly through renal excretion. Approximately 60-70% of the dose was recovered in the urine as active substance. Two metabolites, the mono-desmethyl and the N-oxide, were detected at low concentration in the urine.

Since olopatadine is excreted in urine primarily as unchanged active substance, impairment of renal function alters the pharmacokinetics of olopatadine with peak plasma concentrations 2.3-fold greater in patients with severe renal impairment (mean creatinine clearance of

13.0ml/min) compared to healthy adults. Following a 10mg oral dose in patients undergoing haemodialysis (with no urinary output), plasma olopatadine concentration were significantly lower on the haemodialysis day than on non-haemodialysis day suggesting olopatadine can be removed by haemodialysis.

A renal impairment study after oral dosing of olopatadine has been performed in patients with severe renal impairment. The results indicate that a somewhat higher plasma concentration can be expected with olopatadine in this population. Since plasma concentration following topical ocular dosing of olopatadine are 50-to 200-fold lower than after well-tolerated oral doses, dose adjustment is not expected to be necessary in the elderly or in the renally impaired population. Liver metabolism is a minor route of elimination. Dose adjustment is not expected to be necessary with hepatic impairment.

Indications

OLODIN (Olopatadine Hydrochloride Ophthalmic Solution USP 0.1% w/v) is indicated for the treatment of the signs and symptoms of allergic conjunctivitis.

Recommended Dose

The recommended dose is one drop in each affected eye two times per day at an interval of 6 to 8 hours.

Route of Administration

Ocular

Contraindications

OLODIN (Olopatadine Hydrochloride Ophthalmic Solution USP 0.1% w/v) is contraindicated in persons with a known hypersensitivity to olopatadine hydrochloride or any components of Olodin.

Warnings

OLODIN (Olopatadine Hydrochloride Ophthalmic Solution USP 0.1% w/v) is for topical use only and not for injection or oral use.

Precautions

Information for patients:

To prevent contaminating the dropper tip and solution, care should be taken not to touch the eyelids or surrounding areas with the dropper tip of the bottle. Keep bottle tightly closed when not in use.

Patients should be advised not to wear a contact lens if their eye is red. Olodin (Olopatadine Hydrochloride Ophthalmic Solution USP 0.1% w/v) should not be used to treat contact lens

related irritation. The preservative in Olodin, benzalkonium chloride, may be absorbed by soft contact lenses. Patients who wear soft contact lenses and whose eyes are not red should be instructed to wait at least ten minutes after instilling Olodin (Olopatadine Hydrochloride Ophthalmic Solution USP 0.1% w/v)before they insert their contact lenses.

Carcinogenesis, mutagenesis, impairment of Fertility:

Nursing Mothers

Olopatadine has been identified in the milk of nursing rats following oral administration. It is not known whether topical ocular administration could results in sufficient systemic absorption to produce detectable quantities in the human breast milk. Nevertheless, caution should be exercised when Olodin (Olopatadine Hydrochloride Ophthalmic Solution USP 0.1% w/v) is administered to nursing mother.

Paediatric Use

Safety and effectiveness in paediatric patients below the age of 3 years have not been established.

Geriatric Use

No overall differences in safety or effectiveness have been observed between elderly and younger patient.

Interaction with Other Medicaments

No interaction studies with other medicinal products have been performed.

Side Effects

Headaches have been reported at an incidence of 7%. The following adverse experiences have been reported in less than 5% of patients; asthenia, blurred vision, burning or stinging, cold syndrome, dry eye, foreign body sensation, hyperaemia, hypersensitivity, keratitis, lid edema, nausea, pharyngitis, pruritus, rhinitis, sinusitis and taste perversion. Some of these events were similar to the underlying disease being studied.

Symptoms and Treatment of Overdose

No data are available in humans regarding overdose by accidental or deliberate ingestion. Olopatadine has a low order of acute toxicity in animals. Accidental ingestion of the entire contents of a bottle of olopatadine ophthalmic solution would deliver a maximum systematic exposure of 5mg olopatadine. This exposure would result in a final dose of 0.5mg/kg in a 10kg infant, assuming 100% absorption.

Prolongation of the QT_c interval in dog was observed only at exposure considered sufficiently in excess of the maximum human exposure indication little relevance to clinical use. A 5mg oral dose was administered twice-daily for 2.5days to 102 young and elderly male and female healthy volunteers with no significant prolongation of QT_c interval compared to placebo. The

range of peak steady-state olopatadine plasma concentrations (35 to 127ng/ml) seen in this study represents at least a 70-fold safety margin for topical olopatadine with respect to effects on cardiac repolarisation.

In the case of overdose, appropriate monitoring and management of the patients should be implemented.

Storage Condition

Store at temperature not exceeding 30°C. Protect from light.

After opening the bottle store at below 30°C. Discard contents 28 days after opening the bottle.

Shelf-Life

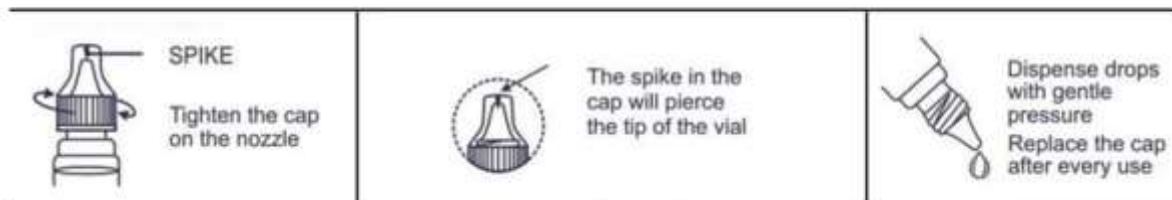
24 months.

After opening: 28 days

Presentation

5.0ml solution filled in 5.0mL of LDPE sealed /closed dropper bottle with a spike HIPS cap. The whole unit is shrink wrapped. Each unit is packed in carton along with package insert.

MODE OF USE



Revised Date

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Manufactured by

FDC Limited

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